



PROTOCOL

FOR THE TRANSITION OF JAMAICAN

ADOLESCENTS LIVING WITH HIV FROM

PAEDIATRIC-BASED TO ADULT-BASED CARE

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Ministry of Health and Wellness

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LIST OF ABBREVIATIONS AND ACRONYMS

ALHIV	Adolescents Living with HIV
ART	Antiretroviral therapy
ARV	Antiretroviral medications
AYALHIV	Adolescents and Young Adults Living with HIV
BHC	Bustamante Hospital for Children
CDC	Centre for Disease and Control
CESD 10	Centre for Epidemiologic Studies Depression Scale
CSO	Community Service Organisation
FGD	Focus Group Discussions
GAD 2	Generalised Anxiety Disorder – 2 question screen
GAD-7	Generalised Anxiety Disorder – 7 question screen
HCP	Health Care Providers
HEADS	Mnemonic for psychosocial review tool
HIV	Human Immunodeficiency Virus
KADS-6	6-Item Kutcher Adolescent Depression Scale
MO	Medical Officer
MOHW	Ministry of Health and Wellness
NGO	Non-Governmental Organisation
NICE	National Institute for Health and Care Excellence
PHQ2	Patient Health Questionnaire – 2 question screening tool
PHQ9-A	Patient Health Questionnaire
PrEP	Pre-exposure Prophylaxis
SERHA	South-East Regional Health Authority
SJPHC	St Jago Park Health Centre
SMART	Social-ecological Model of Adolescent and Young Adult Readiness to Transition
STARx	Successful Transition to Adulthood with Therapeutics = Rx
STH	Spanish Town Hospital
THRxEADS	Mnemonic for psychosocial review tool for adolescents transitioning care
UHWI	University Hospital of the West Indies
UNAIDS	Joint United Nations Programme on HIV/AIDS
UWI	The University of The West Indies
WHO	World Health Organization

INTRODUCTION

The National protocol for Transition of Jamaican Adolescents and Young Adults Living with HIV from Paediatric-based Care to Adult-based Care has at its overarching aim to **promote an equitable, structured, and consistent approach to the preparation, initiation, and completion of the transition process** from paediatric-based care to adult-based care for all Jamaican adolescents and young adults living with HIV (AYALHIVs). The objective is to optimise their continued health and well-being while minimising adult morbidity and premature mortality.

Transition - “*the purposeful, planned movement of adolescents and young adults with chronic physical and medical conditions from child-centred care to adult oriented health care systems*” (1) - has recognized benefits including appropriate expectations of and preparation for the process by all stakeholders, better control of care, and improved outcomes with reduced overall associated costs of healthcare (2).

CONTEXT

In developing the local guidelines for transition of care for Jamaican AYALHIV, the Social-ecological Model of Adolescent and Young Adult Readiness to Transition (SMART) theoretical framework was utilised. The tenets of this framework are supported by the American Academy of Paediatrics (AAP), American Academy of Family Physicians (AAFP) and American College of Physicians (ACP) (White 2018), and the National Institute for Health and Care Excellence (NICE) in the UK (NICE 2016). The adoption of the framework culminated in embracing the following ten principles:

1. Recognition of a **youth-centred approach** with involvement of adolescents and young adults (AYAs) in the development, planning and piloting of the guidelines. *The voices of the youth were actively sought and have directly informed the protocol.*
2. Utilisation of a **strength-based approach** – identifying the unique strengths of each adolescent/young adult and utilising these to strengthen their resilience.
3. **Developmentally appropriate policies:** Adolescence is a complex process with asynchronous but simultaneous changes occurring - all adolescents eventually pass through each phase of adolescence but may not do this in the same way. This implores us to consider the unique developmental characteristics of each AYALHIV during the transition process.
4. Recognition of **common vulnerabilities** as well as unique complexities of each patient (AYALHIV) and utilisation of a **client-focused approach** to ensure that no AYALHIV is left behind to fall through obscured gaps.
5. Emphasising the development of **self-management skills** of the patients. This is crucial for the development of AYALHIVs’ sense of autonomy, self-efficacy, and a sense of personal responsibility regarding their health care needs.
6. **Involvement of a broad range of stakeholders** in the development of the protocol. A wide range of stakeholders were actively involved, hearing the opinions and recommendations of high-level administrative health staff, community service organisations, healthcare providers (physicians, nurses, psychologists, social workers), parents/guardians and the AYALHIVs.

7. Recognising the importance of **preparing AYALHIVs in a timely manner** for transition to adult-based care within the limitations of the local context. The use of a transition guideline which consistently outlines the timing of the transition process itself and relevant reviews across the process will facilitate this.
8. Recruiting **parents/caregivers' engagement** in the transition process as both their role and that of the AYALHIV transform over the transition timeline.
9. Utilisation of **a person-centred approach** with AYALHIVs working together with providers to set and achieve goals together hence optimising adolescents' 'buy-in' and motivation to follow through with attainment of goals.
10. Highlighting the critical component of **communication** between patients, parents, and healthcare providers before, during and after the process of transition. The development of a joint mission statement between paediatric and adult medicine teams will contribute to alignment with this principle.

TRAINING FOR HEALTHCARE PROVIDERS

To successfully implement a transition protocol, health care providers (HCPs) need to understand the transition process (its intentions, the potential challenges and barriers, the facilitators, and its benefits), and secure their buy-in. Therefore, one of the critical steps prior to implementing the programme has been focused training of the HCPs and other associated personnel.

Training should involve didactic presentations, case-based illustrations, role play and other interactive activities with colleagues and with adolescents. Critical modules are:

- HIV Basics (HIV 101)
 - ARV regimes and management
- Developmental characteristics of adolescents (including the HEADSSS approach for evaluation)
- Communicating with the adolescent
- Transition 101– benefits, challenges, barriers, facilitators
- Agile medical management
- Introduction to the protocol guideline
- Resilience-based approach to management
- Preparing the team to deliver psychosocial support
- Psychoeducation of patients and parents/guardians

HEALTH CARE FACILITIES

Clinics should be adequately staffed and equipped to facilitate holistic care for the AYALHIV, inclusive of a HIV-trained physician, nurse manager, social worker, and psychologist. There should be ease of access to a nutritionist, obstetrician, and gynaecologist.

Clinics should have short wait times with specific days/times/sites for AYALHIVs with a robust appointment system to allow for changes and flexibility of appointment times and dates. The facility should be clean, spacious and have allowances for confidential discussion with patients. Staff members engaging with the AYALHIV at all levels (ancillary to executive) must always be respectful and non-judgmental in their interactions with AYALHIVs.

GUIDED HEALTHCARE VISITS FOR THE AYALHIV

General well care of the AYALHIV should be consistent with recommendations for all adolescents to ensure optimal overall health and wellness. Follow-up visits for AYALHIVs are recommended every 3-6 months depending on their status and health outcomes, with more frequent visits as required for at-risk clients.

At each visit a history should be taken regarding any concerns the patient and or parent/guardian have since their last visit. This should be followed by enquiry regarding any concerns or comorbidities identified at the previous visit(s).

Developmental progress enquiry should be performed regarding pubertal changes, progress at school, age-appropriate self-help skills. This assessment should be done with consideration of the AYALHIV's developmental and not chronological age.

Enquiry regarding social support and home environment inclusive of screening for interpersonal violence, abuse/neglect, community support/violence should be included at each visit.

Psychosocial review should be performed at each visit by the HCP, and this is recommended with utilising two mnemonics – HEADS (3) and its adaptation for adolescents in the process of transition, THRxHEADS (4). Table 1 below which combines content of both mnemonics for ease of use.

Table 1. Modified THRxEADS – psychosocial review for adolescents transitioning care for a chronic illness.

Transition	Can you explain the important things about your condition in three or four sentences? How much of your health care is your responsibility? What do you need to do to get ready for adult health care? What kind of services will you need to find in the adult system?
Home	Where do you live? Who do you live with? How well do you get along with the people you live with? Anyone at home you can talk to? Recent additions/ losses? Are there any issues with housing and your health? (accessibility/allergens/others) Do you have concerns about how your illness affects your family's function? Who knows about your illness? (family members/friends) If you live in more than one home, are there differences in the way you take care of your illness in the two homes?
Rx - Medication and Treatment	What medications do you take? What are the dosages? Who is in charge of your medication? What is your role? How do your medications/treatments fit in your routine during the week/weekend? What happens when you miss a dose? What do you like and dislike about your medication?
Education, Eating, Employment	Do you go to school? What school? What grade are you in? Ever skipped/repeated a grade? Are you in a special class? Friends at school? Recent changes in school? Ever suspended/expelled? Adults at school you can talk to? Are you working? Where? How much? Future plans for work/ school? Does your illness make things different or difficult for you at school? Is there someone you can talk to or go see for help/support at school? How often do you have to miss school because of your illness? Any recent changes in your weight? Have you dieted in the past year? How often? Exercise? Anything else to manage your weight? What do you think is a healthy diet? How does yours compare? What do you like/dislike about your body? How does your condition or medications affect your diet and appetite? Does your condition affect how your body looks? In what way?

Activities, Affect	Do you have friends? What do you and your friends do for fun? What do you and your family do for fun? Extracurricular activities? Hobbies? Media time/ internet safety? Does your condition get in the way of participating in activities that your friends do? Do you have friends or talk to people who have similar health/disability issues? Do you use the internet to connect with other teens with health conditions or learn about your condition? Does your condition affect your sleep? Does your condition make you sad, angry or anxious? Have you ever had suicidal thoughts or tried to hurt yourself because of your condition?
Drugs	Do any of your friends smoke/ use alcohol or other drugs? Do you smoke? Drink? If yes, how much and how often? Anyone in your family smoke/drink/ use other drugs? Drink or use other drugs alone? CRAFFT questions (see Box 5) What do you know about the effect of alcohol/cigarettes/street drugs on your condition or treatments? Do you ever use alcohol/cigarettes/street drugs to treat your condition, symptoms or medication side effects?
Sexuality, Safety, Suicide, Sleep	Are you/ have you been in a romantic relationship? Have you ever had sex? Are you using birth control? Do you use condoms every time? Have you ever been pregnant/ Gotten someone pregnant? Did anyone ever make you do something you didn't want to do? How many partners have you had? Have you ever had an STI? What do you know about the effect of your condition on your sexual health? Are there any limits on contraception you can use because of your condition? Have you talked about the genetics of your condition? (i.e., can it be passed on to your children?) Do you ever feel sad or down more than usual? What do you do to cheer yourself up? Do you ever think a lot about or want to hurt yourself/ someone else? Have you ever tried to kill yourself? Do you have anyone to talk to? Who? Trouble sleeping? How difficult is it to fall asleep/ stay asleep?

All ALHIVs should be screened for symptoms of anxiety and depression starting at age 10 years. This should be incorporated into **each** visit using the Generalised Anxiety Disorder – 2 question screen (GAD-2) and the Patient Health Questionnaire – 2 question screen (PHQ-2) shown below in Box 1 and Box 2 respectively.

Box 1. Generalised Anxiety Scale - 2 (GAD-2)

Over the last 2 weeks, how often have you been bothered by the following problems?				
(Tick “✓” the one that is correct for you)				
	0 Not at all	1 Several days	2 More than half the days	3 Nearly every day
1. Feeling nervous, anxious, or on edge				
2. Not being able to stop or control worrying				

Box 2. Patient Health Questionnaire - 2 (PHQ-2).

Over the last 2 weeks, how often have you been bothered by the following problems?				
(Tick “✓” the one that is correct for you)				
	0 Not at all	1 Several days	2 More than half the days	3 Nearly every day
1. Little interest or pleasure in doing things				
2. Feeling down, depressed, or hopeless				

If patients screen positive (score ≥ 3) on either scale, they should be asked to complete the GAD-7 (Box 3) and PHQ-9 for adolescents (Box 4) with review of same by the physician, and further exploration of symptoms and associated factors through a clinical interview. An alternate tool is the KADS-6 in Box 5.

Box 3. Generalised Anxiety Disorder – 7 question screen (GAD-7)

Over the last 2 weeks, how often have you been bothered by any of the following problems?

	0 NOT AT ALL	1 SEVERAL DAYS	2 MORE THAN HALF THE DAYS	3 NEARLY EVERY DAY
1. Feeling nervous, anxious or on edge?				
2. Not being able to stop or control worrying?				
3. Worrying too much about different things?				
4. Having trouble relaxing?				
5. Being so restless that it is hard to sit still?				
6. Becoming easily annoyed or irritable?				
7. Feeling afraid as if something really bad might happen?				
<i>Spitzer RL, Kroenke K, Williams JBW, Lowe B. A brief measure for assessing generalized anxiety disorder. Arch Internal Medicine 2006 166:1092-1097.</i>				

Total Score	Anxiety Severity
0 to 4	Minimal anxiety symptoms
5 to 10	Mild anxiety symptoms
10 to 14	Moderate anxiety symptoms
15 to 21	Severe anxiety symptoms

Box 4. Patient Health Questionnaire - 9 question screen for adolescents (PHQ9-A)

How often have you been bothered by each of the following symptoms during the **past two weeks**? For each symptom put an "X" in the box below the answer that best describes how you have been feeling.

	(0) Not At All	(1) Several Days	(2) More Than Half the Days	(3) Nearly Every Day
1. Feeling down, depressed, irritable, or hopeless?	☐	☐	☐	☐
2. Little interest or pleasure in doing things?	☐	☐	☐	☐
3. Trouble falling asleep, staying asleep, or sleeping too much?	☐	☐	☐	☐
4. Poor appetite, weight loss, or overeating?	☐	☐	☐	☐
5. Feeling tired, or having little energy?	☐	☐	☐	☐
6. Feeling bad about yourself – or feeling that you are a failure, or that you have let yourself or your family down?	☐	☐	☐	☐
7. Trouble concentrating on things like school work, reading, or watching TV?	☐	☐	☐	☐
8. Moving or speaking so slowly that other people could have noticed? Or the opposite – being so fidgety or restless that you were moving around a lot more than usual?	☐	☐	☐	☐
9. Thoughts that you would be better off dead, or of hurting yourself in some way?	☐	☐	☐	☐
In the past year have you felt depressed or sad most days, even if you felt okay sometimes? ☐ Yes ☐ No				
If you are experiencing any of the problems on this form, how difficult have these problems made it for you to do your work, take care of things at home or get along with other people? ☐ Not difficult at all ☐ Somewhat difficult ☐ Very difficult ☐ Extremely difficult				
Has there been a time in the past month when you have had serious thoughts about ending your life? ☐ Yes ☐ No				
Have you EVER , in your WHOLE LIFE, tried to kill yourself or made a suicide attempt? ☐ Yes ☐ No				

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Guidelines for Adolescent Depression in Primary Care (GLAD-PC)

Total Score	Depression Severity
0-4	No or Minimal depression
5-9	Mild depression
10-14	Moderate depression
15-19	Moderately severe depression
20-27	Severe depression

Box 5. 6-ITEM Kutchner Adolescent Depression Scale (KADS-6)

6-ITEM Kutchner Adolescent Depression Scale: KADS-6				
NAME: _____		CHART NUMBER: _____		
DATE: _____		ASSESSMENT COMPLETED BY: _____		
OVER THE LAST WEEK, HOW HAVE YOU BEEN "ON AVERAGE" OR "USUALLY" REGARDING THE FOLLOWING ITEMS:				
1. Low mood, sadness, feeling blah or down, depressed, just can't be bothered.				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
2. Feelings of worthlessness, hopelessness, letting people down, not being a good person.				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
3. Feeling tired, feeling fatigued, low in energy, hard to get motivated, have to push to get things done, want to rest or lie down a lot.				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
4. Feeling that life is not very much fun, not feeling good when usually (before getting sick) would feel good, not getting as much pleasure from fun things as usual (before getting sick).				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
5. Feeling worried, nervous, panicky, tense, keyed up, anxious.				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
6. Thoughts, plans or actions about suicide or self-harm.				
☐	☐	☐	☐	
0 - Hardly Ever	1 - Much of The Time	2 - Most of The Time	3 - All of The Time	
TOTAL SCORE:		1		
© Dr Stan Kutcher, 2006				

KADS-6 is available for open use. No copyright restrictions.

Scoring of the 6-item Kutchner Adolescent Depression Scale (KADS)

Add all 6 item scores to form a single total score.

Total scores at or above 6 suggest "possible depression" (and a need for more thorough assessment).

Total scores below 6 indicate "probably not depressed."

All AYALHIV should also be screened for any features of self-harm (e.g., 'cutting'), as well as suicidal thoughts and plans at each visit, regardless of whether assessed as being depressed or not, and should be referred to the psychologist or psychiatrist when identified (see Box 6 below).

Box 6. Suicide Screening ASQ



Ask Suicide-Screening Questions

Ask the patient:

(1) In the past few weeks, have you wished you were dead?	YES	NO
(2) In the past few weeks, have you felt that you or your family would be better off if you were dead?	YES	NO
(3) In the past week, have you been having thoughts about killing yourself?	YES	NO
(4) Have you ever tried to kill yourself?	YES	NO
If yes, how? _____ When? _____		

If the patient answers yes to any of the above, ask the following question:

(5) Are you having thoughts of killing yourself right now?	YES	NO
If yes, please describe: _____		

Horowitz LM, Bridge JA, Teach SJ, et al. Ask Suicide-Screening Questions (ASQ): a brief instrument for the pediatric emergency department. Arch Pediatr Adolesc Med. 2012;166(12):1170-1176. doi:10.1001/archpediatrics.2012.1276

It is not recommended that you ask number 4 again if the answer has previously been 'yes'.

All AYALHIV should be routinely screened for substance use. This can be done using the CRAFFT screening tool which has been validated for use in adolescents with a cut-point of 2 (see Box 7 below).

Box 7. CRAFFT Screening Tool for Substance Misuse

C - Have you ever ridden in a **CAR** driven by someone (including yourself) who was "high" or had been using alcohol or drugs?

R - Do you ever use alcohol or drugs to **RELAX**, feel better about yourself, or fit in?

A - Do you ever use alcohol/drugs while you are by yourself, **ALONE**?

F - Do you ever **FORGET** things you did while using alcohol or drugs?

F - Do your family or **FRIENDS** ever tell you that you should cut down on your drinking or drug use?

T - Have you gotten into **TROUBLE** while you were using alcohol or drugs?

Mitchell, S. G., Kelly, S. M., Gryczynski, J., Myers, C. P., O'Grady, K. E., Kirk, A. S., & Schwartz, R. P. (2014). The CRAFFT cut-points and DSM-5 criteria for alcohol and other drugs: a re-evaluation and re-examination. Substance abuse, 35(4), 376–380.

The AYALHIV may be screened for Post-Traumatic Stress Disorder, which can be done using the PC-PTSD-5 tool shown below in Box 8.

Box 8. Primary Care PTSD Screen for DSM-5 (PC-PTSD-5)

Primary Care PTSD Screen for DSM-5 (PC-PTSD-5)

Sometimes things happen to people that are unusually or especially frightening, horrible, or traumatic. For example:

- a serious accident or fire
- a physical or sexual assault or abuse
- an earthquake or flood
- a war
- seeing someone be killed or seriously injured
- having a loved one die through homicide or suicide.

Have you ever experienced this kind of event?

YES NO

If no. Please stop here.

If yes, please answer the questions below.

In the past month, have you...

1. had nightmares about the event(s) or thought about the event(s) when you did not want to?

YES NO

2. tried hard not to think about the event(s) or went out of your way to avoid situations that reminded you of the event(s)?

YES NO

3. been constantly on guard, watchful, or easily startled?

YES NO

4. felt numb or detached from people, activities, or your surroundings?

YES NO

5. felt guilty or unable to stop blaming yourself or others for the event(s) or any problems the event(s) may have caused?

YES NO

Scoring

A total score is determined as a sum of responses to all 5 questions (yes =1; no =0)

Validation studies indicate a positive screen for PTSD with a score of 3 or more (Donenberg, G., et al., (2021). *Mental health outcomes of a pilot 2-arm randomized controlled trial of a HIV-prevention program for South African adolescent girls and young women and their female caregivers*. BMC public health, 21(1), 2189).

A fulsome physical examination should be completed at each visit inclusive of anthropometric measurements (height, weight, BMI) and plotted to assess growth velocity, vital signs (pulse, blood pressure, respiratory rate), examination of each system including the skin. A dental exam, pubertal assessment and screening for scoliosis should be completed at each visit. Females should have a breast examination and males a testicular examination annually. Proposed documentation for (i) new patients and (ii) follow-up visits are included as Appendix 4 (*Intake form – New adolescent patient*) and Appendix 5 (*Follow-up adolescent patient – Clinic Intake form*) respectively.

IDENTIFICATION OF HIGH-RISK PATIENTS

AYALHIVs may be at high risk from multiple perspectives: **medical risk factors** – comorbidities, poor adherence, HIV-related neurocognitive deficits; **psychosocial risk factors** – limited or no emotional support, history of trauma, mental health disorder, school difficulties; **financial risk factors** – food insecurity; unemployable. At the end of completing the full assessment, those adolescents deemed as high-risk should be flagged for closer monitoring and review by the management team, and customised case conferences should be convened.

All 'high risk' patients should be discussed at an informal team meeting of team lead, physician, nurse, psychologist, social worker **within 2 weeks** of identification. High risk cases are defined as AYALHIVs with:

- **worsening medical concerns** - increasing viral loads, ARV treatment failure resulting in progression of medication regimens e.g. 1st to 2nd or 2nd to 3rd line medications.
- **psychosocial concerns** - reporting symptoms of significant emotional distress (elevated GAD-2 or PHQ2, self-harm, active suicidal ideation, suicide attempt).
- **recurrent missed appointments** (≥2 missed consecutive appointments).

Timely referrals to the social worker, psychologist, behaviour-change specialist, and other health professionals/medical sub-specialists should be made as deemed fit.

DISCLOSURE

A requirement for transitioning to the next level of clinic care should be disclosure of diagnosis to the ALHIV prior to transfer. Based on patient, parent/guardian and healthcare provider feedback, **age 12 years is the best time to have completed this process** with the required supportive framework in place. **The recommended time to initiate the process is approximately 8 years of age.** Disclosure has been shown globally and locally to have a profound effect on AYALHIVs' self-management and health outcomes and is therefore a critical and success-defining criteria.

OPTIMISING THE DISCLOSURE PROCESS

International literature and local data collected have identified the process of disclosure of ALHIV's diagnosis as a limiting step in the process of acceptance, adherence to medications and engagement in self-management. Optimal management of the disclosure process is highlighted in this proposed transition protocol as a critical step. The recommended process for disclosure of the HIV diagnosis is outlined in Table 2 below (adapted from the algorithm found in *"Guidelines on use of Antiretroviral Drugs for Treating and Preventing HIV Infection in Kenya 2016; p143"*) (5).

Table 2. Age-appropriate disclosure for children and adolescents living with HIV.

Age (years)	Primary Objective	Tasks for provider to complete
0-6	No disclosure	Inform parents will start partial disclosure at age 7y Make disclosure plan with parent/guardian Explain process of disclosure to parents – explore any fears or concerns they may have and engage in relevant psychoeducation
7-9	Partial disclosure	Introduce basic concepts i) HIV as a germ ii) CD4 cells - help to fight war against germs iii) Drugs – taking medication to stay healthy Review disclosure plan with parents/caregivers Refer parents for psychological support if experiencing any persistent concerns or fears regarding disclosure process Enhance parent/guardian's HIV literacy

Age (years)	Primary Objective	Tasks for provider to complete
10-12	Full disclosure	Disclosure 1) Assess level of social support to ensure sufficient in period following disclosure. 2) Determine ALHIV's level of knowledge about HIV/AIDS from prior exposure at school. Enquire of myths and fears and offer correct advice and reassurance as necessary. Reinforce accurate information. 3) Assess ALHIV's level of reaction to potential disclosure with use of a story and utilise this feedback to plan for impending disclosure. 4) Complete actual disclosure to patient – providers can support parents/caregivers as they disclose or may disclose to the patient directly as preferred by the parent/caregiver and decide upon by joint effort of provider and parent. Respond to any immediate concerns the adolescent may have and offer support. 5) Assess the need for referral to psychologist within 2-3 weeks post-disclosure and arrange as required. Provide parental support for the process of full disclosure.
13-15y	Development of health literacy post-disclosure	Help ALHIV understand how meds work, timing, etc. Refer to ALHIV individual/group therapy sessions as needed

PREPARING AYALHIV, PARENTS AND HCPS FOR TRANSITION

Preparing all stakeholders for the process of transition of AYALHIVs from paediatric-based to adult care requires fulsome involvement of all stakeholders – patient, parent, and providers. An algorithm for guiding the transition process (adapted from the Adolescents package of care in Kenya) (5) is outlined below in Figure 1.

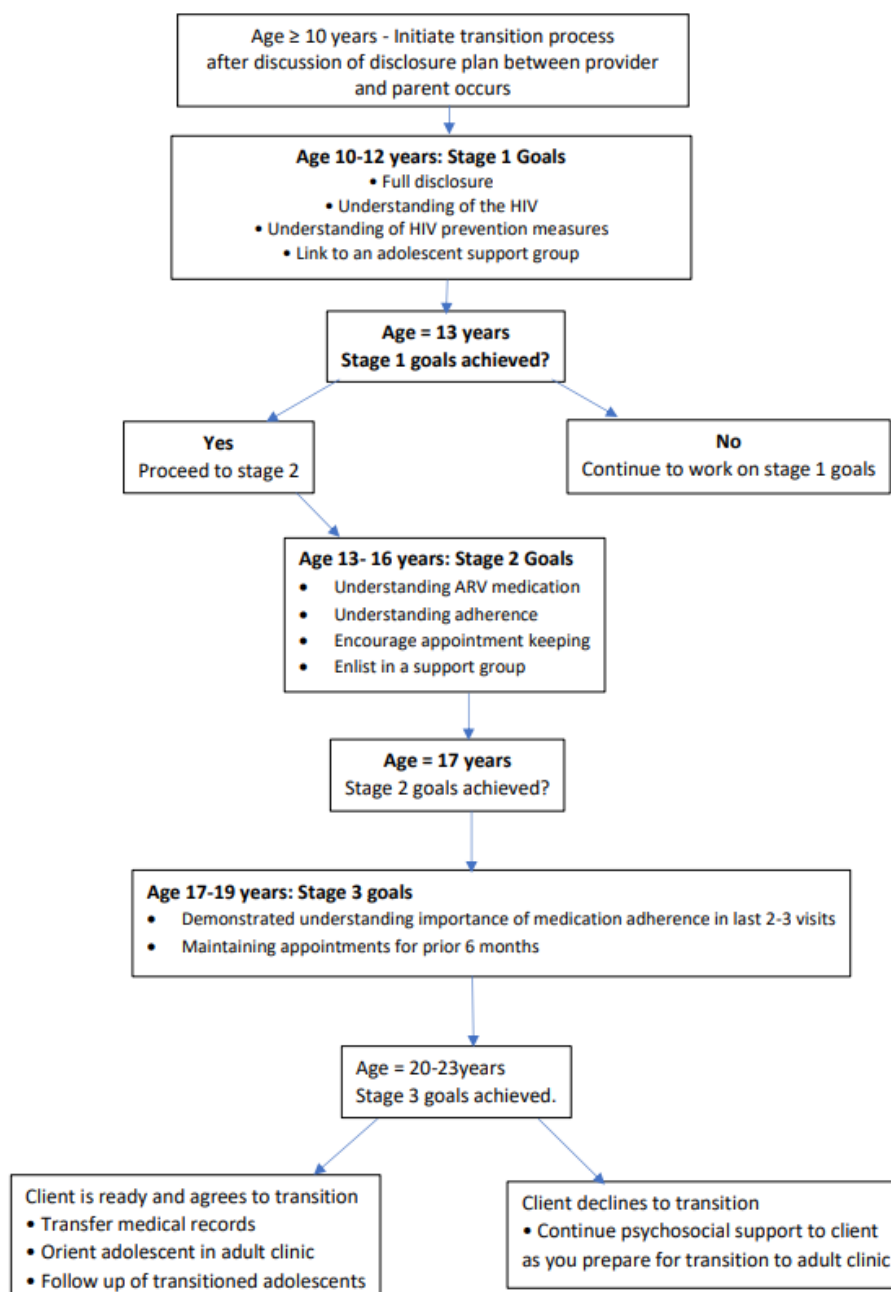


Figure 1. Algorithm for transition process.

Emotional and psychological support must be afforded to parents/ guardians when needed as they proceed through the transition process. Parent group support sessions monthly have been lauded by parents locally as very helpful to the process.

Steps to be completed by the HCP at each phase of the process are itemised in Table 3 below.

Table 3. Overall guideline for ALHIV transition procedure

Age (years)	Primary Objective	Tasks	Tick when completed	Comment(s)
0-6	No disclosure	● Inform parents will start partial disclosure at 6y ● Make disclosure plan with parent/guardian		
7-9	Partial disclosure	● Introduce basic concepts i) HIV as a germ ii) CD4 cells - help to fight war against germs iii) Drugs – taking medication to stay healthy ● Review disclosure plan with parents/caregivers ● Enhance parent/guardian's HIV literacy		
10-12	Full disclosure	● Disclosure ● Assess level of social support to ensure sufficient following disclosure ● Determine ALHIV's level of knowledge about HIV/AIDS from prior exposure at school. ○ Enquire of myths and fears and offer correct advice and reassurance as necessary. ○ Reinforce accurate information ● Assess ALHIV's level of reaction to potential disclosure with use of a story and utilise this feedback to plan for impending disclosure ● Complete actual disclosure to patient ○ Providers should support parents/ caregivers as they disclose or may disclose to		

Age (years)	Primary Objective	Tasks	Tick when completed	Comment(s)
		the patient directly as decided in discussion between provider(s) and parents/guardians. ○ Respond to any immediate concerns the adolescent or parent may have post-disclosure and offer support and referral as needed. ● Assess need for referral to psychologist within 2-3 weeks post-disclosure and arrange as required. ● THRxEADS psychosocial review at each visit ● Mental health screening at each visit (GAD-2; PHQ-2) ● Advise adolescent of proposed time for move to new clinic at least 1 year prior to transfer. ● Engage adolescent regarding preference for new clinic to be transferred to at least 6-12 months prior to moving ● Arrange visit to new clinic at least once before transfer (BHC; STH)		
13-16y	Development of health literacy and self-management skills	● Help ALHIV understand how meds work ● Achieve adherence ● Engage in making and keeping appts ● THRxEADS review at each visit ● Mental health screening at each visit (GAD-2; PHQ-2) ● Advise adolescent of proposed time for move to new clinic at least 1 year prior to transfer - age 14y (STH); 20-24y (SJPHC; UHWI) ● Engage adolescent regarding preference for new clinic to be transferred to at least 6-12 months prior to moving (STH) ● Arrange visit to new clinic at least once before transfer (STH)		

Age (years)	Primary Objective	Tasks	Tick when completed	Comment(s)
		- Initiate discussions regarding high-risk behaviours – delaying sexual initiation, sexual risk-taking, risk of abuse, healthy relationships; contraception - Refer to ALHIV group therapy sessions		
17-19y	Prepare for transition	- Continue discussions re healthy relationships, delay of sexual initiation, contraception, risk of abuse - Initiate discussion re pre-exposure prophylaxis (PrEP) for sexual partners <i>(PrEP refers to medicine taken before sexual intercourse to prevent getting HIV. It is highly effective for preventing HIV if taken as prescribed.)</i> - PrEP reduces the risk of getting HIV from sex by about 99%. - PrEP reduces the risk of getting HIV from injection drug use by at least 74%. - Continue group sessions – self-management, coping strategies, disclosing to friends/partners - Identify new adult HCP/ contact in new clinic 6 months prior to transfer - Transition Summary - written 3 months prior to transfer (see <i>Appendix 1</i>) - Arrange visit to adult clinic at least once before transfer - Meet adult HCP / clinic staff member at least once prior to transfer - Complete transition checklist (see <i>Appendix 2</i>)		

IMPROVING HEALTH LITERACY

Health literacy is defined as ‘the degree to which individuals have the capacity to obtain, process, and understand basic health information and services needed to make appropriate health decisions (6, 7).

It is an important public health issue. While much research has examined the quality of health literacy and impact on health outcomes for adults, little is known about health literacy for adolescents.

From the perspective of prevention, one can assume that inculcating health literacy during adolescence will lead to the individual’s understanding of health issues and the interaction with the health care system with resultant positive health outcomes in adulthood. While this approach is broadly applicable to all adolescents, those with chronic diseases for example HIV should specifically be targeted for management. Adolescents with chronic diseases have more interaction with the health care system and greater participation in and responsibility for their treatment and care.

HEALTH LITERACY GOALS

The following are key minimal goals and expectations for adolescent health literacy:

- Completing medical forms.
- Understanding instructions for medications and treatment.
- Providing accurate history of complaints and other clinical problems.
- Participating in health education programmes (media, campaigns, etc.) - read, understand, access messages.

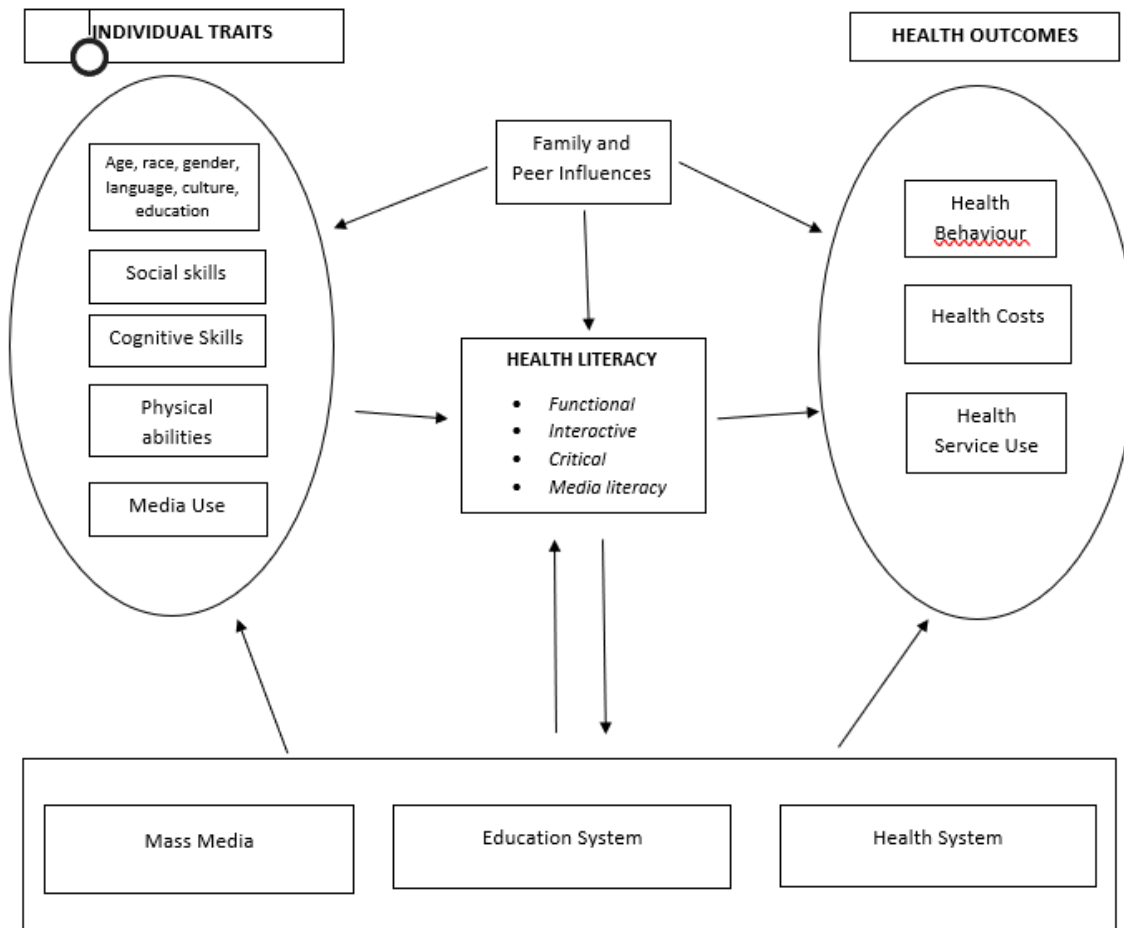
ADOLESCENT CONCERNS

Healthcare workers must be cognisant of the challenges associated with achieving health literacy in adolescents. These include:

- Low levels of literacy skills at appropriate grade level.
- Adolescent development spectrum - and capacity to understand, process and evaluate health information; difficult for 25% of adolescents (8, 9).
- Readability of educational material - appropriateness for age.
- Complexity of the crucial stage of development for adolescents (physical, emotional, and cognitive changes).

Cognitive development during adolescence progresses towards improved capacity for processing information, thinking more about abstract ideas and using reasoning skills; achieving greater autonomy. It is therefore ideal to target adolescents to begin health literacy interventions (10). Hopefully they will acquire the knowledge and adopt appropriate health seeking behaviours as they transition to adulthood.

The conceptual framework (Figure 2) for assessing adolescent health literacy should consider individual characteristics, peers/parental influences, and the systems with which they co-exist (7).



Adapted from: Manganello JA. Health literacy and adolescents: a framework and agenda for future research. *Health education research*. 2008;23(5):840-7.

Figure 2. Conceptual framework for studying adolescent health literacy.

INDIVIDUAL

Individual traits, such as age, race, gender, cultural background, cognitive and physical abilities, and social skills predict one's health literacy which then determines health outcomes (6).

Media use is a crucial component of an adolescent-specific framework given that adolescents are frequent users of all forms of social media (11). Furthermore, media has been shown to influence physical and social development, health behaviour, and literacy (12).

PEER AND PARENT INFLUENCES

Parents/guardians and peers can directly influence health literacy and behaviour of adolescents. It is also therefore important to determine the quality of parental/guardian health literacy and optimise to impact the outcomes further positively for adolescents (6).

SYSTEMS

The health system, education system, culture, and society all potentially contribute to the development of health literacy skills, and therefore can be targeted for interventions.

One of the main goals of the education system is to develop literacy skills. Health literacy skills (which includes media literacy and critical thinking) are necessary for adolescents to be able to access health information from mass media, understand the content and evaluate the credibility of the information they obtain.

The health system is clearly important to include in a framework of health literacy, given its direct impact on how accessible health care is and how information is communicated to patients (6).

Readability of materials in health centres, clarity of provider communication with adolescent patients and interventions in health settings are all crucial to the development of health literacy skills for adolescents.

Whereas educational programmes implemented in schools/health facilities, or conducted via social media, can teach adolescents about health issues, the success of such interventions is dependent on the target groups having access to and the ability to understand the messages being provided.

MEASUREMENT OF HEALTH LITERACY IN ADOLESCENTS

There is no definitive, or clear definition of what exactly is to be measured when assessing the health literacy of a child, not only in terms of subject content and knowledge, but also in terms of levels (functional, interactive and critical) of literacy, types of literacy (prose, numeracy and document), abilities and skills (seek, understand, communicate and evaluate) (13).

Most instruments tend to measure their own specific understanding of health literacy and not all provide sufficient conceptual information. To advance health literacy instruments, a much more standardised approach is necessary including improved reporting on the development and validation processes (14).

Manganello and colleagues proposed the Health Literacy Assessment Scale for Adolescents (HAS-A). This is a self-report tool using items in sub scales of Communication, Confusion and Functional Health Literacy. Its utility however is for research (15). Further study is needed to identify simple and short tools to be used in a busy outpatient adolescent health setting.

PSYCHOEDUCATION FOR AYALHIVs AND THEIR PARENTS/GUARDIANS

Psychoeducation refers to the process of sharing information and education about an illness/disorder with the aim being to educate the recipient about the nature and treatment of the illness/disorder. This can be delivered in a cost-effective manner by the clinic-based health care provider, typically nurse or physician (16).

The Health Belief Model proposed almost three decades ago speaks to patients balancing their own perceptions of the likelihood of an adverse outcome and its attendant consequences with the pros and cons of embracing and adopting a protective behaviour (17). Health care providers often respond to a patient's suboptimal adherence to a regime by re-emphasising biomedical facts about the illness, but may have more far reaching impact by considering other factors in addition to disease knowledge (16). In other words, **a patient's motivation to be adherent with their ARV medications may be influenced by their knowledge and understanding of medications and the illness but also influenced by how the patient experiences the illness in their unique context.** Laws and colleagues suggest that non-biomedical approaches to explanations may be more impactful in helping persons in their decision making around their illness (16). Health care providers will be encouraged to review regularly and in a simple way, the basics of HIV and ARVs, while getting the perspective of the ALHIV on the pros and cons of the illness and the medications.

It is crucial for HCPs to consider the developmental perspective, as these AYALHIV are traversing that phase of life of identity vs role confusion (Erikson's psychosocial stage 5) and consider how this may influence how they experience the illness, even as they live with the associated stigma whether self-imposed and/ or externally applied. As adolescents experience await further development of their prefrontal cortex, they may experience impaired ability to appreciate long term consequences of their decisions secondary to suboptimal decision-making capabilities.

Health care providers must therefore utilise **a developmentally sensitive, informed yet non-judgmental approach to their psychoeducational efforts with AYALHIVs** in order to get maximal output.

Laws and colleagues developed an instrument to investigate i) ARV adherence ii) need for cognition (decision making through technical understanding of HIV disease and treatment) iii) general feelings about living with HIV and ARV medications iv) reasons for taking ARVs v) reasons for not taking ARVs and vi) HIV knowledge quizzes (16). Use of the instrument facilitated identification of PLHIVs' gaps in knowledge about the illness and their varied misconceptions regarding the illness and the recommended treatment. This information would give the HCP team the opportunity to develop transition plans for each AYALHIV recognizing their unique strengths and challenges in a holistic manner.

SELF-MANAGEMENT IN AYALHIVs

A CRITICAL COMPONENT IN TRANSITION OF CARE

A successful transition process is in part due to good self-management by AYALHIVs. Self-management is not optional but inevitable and considers a patient's current practices and goals for adherence, immediate needs, personal interest, psychological state, and personal, social, and medical barriers. Patients are required to comply with instructions and assume an active role in medical decisions about their treatment. To maintain good health as well as a successful transition process, AYALHIVs must learn the skills needed to cope with living with HIV as well as those needed to manage the emotional, physical, and practical issues that may arise. By doing so, AYALHIVs may engage in medical management (e.g. taking medication as prescribed), behavioural management (making lifestyle changes that consider HIV), and emotional management (dealing with emotional reactions about HIV-related concerns, be it from others or self).

As they transition to adult care, AYALHIVs are expected to make decisions about their lives by garnering knowledge about HIV, its treatment, and what support systems are available. Knowing how and when to make use of that knowledge is crucial to self-management as it facilitates AYALHIVs speaking up for themselves or, at least, learning how to do so. Knowing yourself, knowing your needs, and knowing how to get what you need are crucial to becoming an effective self-advocate. This skill, self-advocacy, can allow AYALHIVs to get the best care possible and foster self-management.

MEDICAL MANAGEMENT

Health literacy and subsequent adherence to medication impact patients' self-management as these are crucial to the maintenance of AYALHIVs' good health and should be promoted. However, medication regimens may be complex and, as a result, may threaten effective self-management. The lack of direct self-monitoring of physical status is an additional contributing factor as the progression of HIV may only be observed through tests and correspondence with the AYALHIV's healthcare provider (18). AYALHIVs in Jamaica as interviewees described poor self-management skills for adherence to antiretroviral medication, partly due to their lack of understanding of the results of tests done. This, in turn, elucidates the need for effective communication between AYALHIVs and their healthcare providers, self-advocacy, and self-care.

BEHAVIOURAL MANAGEMENT

Self-care starts with taking responsibility for making lifestyle choices daily. AYALHIV may choose to take medication as prescribed, have safe sex, use condoms correctly, limit the number of sexual partners, not share needles, consider PrEP for sexual partner(s), or not. The behaviours an individual engages in may be healthy or unhealthy, however, AYALHIVs being mindful of lifestyle choices underscores their self-management and promotes healthy living and a successful transition process. Resources and knowledge are available to AYALHIV and should continue to be promoted. The seeking of these resources and knowledge is advantageous in AYALHIVs management of self.

Living with any chronic illness may prove difficult to accept as realising lifestyle choices may be influenced by emotions. Given the negative perceptions of the HIV diagnosis being commonplace in the Jamaican society, the emotional concerns of AYALHIVs in the Jamaican context are predominantly enslaved to the impact of stigma. This stigma surrounding HIV acts as a barrier to healthcare.

Interviewees noted their apprehension to visit the adult clinic as it ensures the potential for unwanted interactions between themselves and a person with whom they are familiar. This apprehension is bolstered by feelings of embarrassment and discomfort due to feeling trapped in situations in which they may be led to disclose their HIV status. To not face these feelings, AYALHIVs may opt to not attend the clinic which, may in turn, negatively impact their health. However, there are those AYALHIVs that may choose to travel farther to attend clinic to lessen this potentiality, despite the cost. Transportation and finances are notable social domains that have an impact on self-management. Specifically, getting to and from appointments may be challenging for some AYALHIVs since the cost of transportation may be too expensive. Though antiretroviral medication and blood testing may also be costly, some AYALHIVs may opt to outsource these resources to private entities to deter stigma, while others may struggle to fund their treatment.

Access to mental health services has also been noted as an area of particular interest for AYALHIVs in the Jamaican context. Though mental health services are promoted to AYALHIVs, access to these services may be limited or restricted due to financial limitations. Challenges related to cost and transportation arise, yet again, should these services be offered offsite. Interviewees also noted their interest in continued mental health group sessions at adult clinics, suggesting a decline in such services in some adult clinics.

Individuals' external environments and experiences impact their emotional functioning. Hence, other challenges in psychosocial/emotional management are due to negative intrapersonal feelings and attitudes surrounding the diagnosis of HIV and the phenomenology of its experience. That is, in addition to the effects of living with a chronic illness, AYALHIVs' experiences put them at greater risk of developing mental health conditions, oftentimes depression and anxiety (19). Hence, managing emotions is an impacting factor in self-management.

Intrapersonal and intrapsychic experiences also impact emotional functioning as individuals develop through the lifespan. AYALHIVs are in a period of life in which they are (becoming) capable of having/garnering a clearer understanding of what HIV is and what is involved in its treatment. Simultaneously, in an effort to gain autonomy, which is especially crucial in late adolescence to young adulthood (18-24-years-old) (20), AYAs playing a role in their own health becomes relevant as it fosters their personal development. Notably however, maturity is not suddenly completed on becoming 18 years old, and late adolescents and young adults alike, need continued support as they transition into a more self-reliant period of life.

SELF-MANAGEMENT IDENTIFICATION AND RESPONSE

Though achieving balance in life is not an impossible task, it is a challenging one. Hence, individuals may do the best that they can to manage their health even while leaving room for further improvement. The AYALHIV who are managing themselves well may be doing so by engaging in good medical, behavioural, and psychosocial/emotional management as well as being a self-advocate. That is, such patients may be: adhering to their ART regimen; going to clinic visits as scheduled; clearly communicating their diagnosis and presenting concerns to healthcare providers; seeking information and clarity wherever needed; having safe and responsible sex; considering PrEP for their sexual partner(s); and seeking support from friends, family, medical care providers, and mental health care providers.

As informed by the lived realities of the interviewees and that noted by DeSouza et al previously (21), those AYALHIVs need to meet the following criteria to indicate a minimal self-sufficient level of self-management in the transition process:

- adherence to the ART regimen;
- consistent attendance at clinic visits;
- adherence to scheduled clinical investigations;
- clear communication of diagnosis, status, and current treatment regimen;
- communication of any difficulties (eg side effects) being experienced with their provider
- consistent attendance at mental health sessions, be it group or individual; and
- clear understanding of safe sex practices.

These criteria inform substantial self-care, engagement, self-advocacy, and decision-making skills.

INCREASING SELF-MANAGEMENT IN AYALHIVS

In addition to the various aspects of self-management that AYALHIVs must work on improving, self-management is also fostered by support and a conducive environment. Hence, considerations must be made for ways in which parents/guardians can contribute to the betterment of AYALHIVs' self-management. Some parents/guardians should be offered social support where needed, be it due to poor parenting, a poor parent-child dynamic, and/or ignorance of the multifaceted characteristic of social support. Similarly, the impact of healthcare providers' roles in treatment must be considered.

Social support is crucial to the management of self and is not divorced from chronic illness, including HIV. It is a category system consisting of five categories: **emotional, instrumental/tangible, informational, appraisal/esteem, and social network support** (22). **Emotional support** is related to the expressions of empathy, sympathy, love, trust, and care. **Instrumental/tangible support** refers to the provision of needed goods and services. **Informational support** refers to the provision of knowledge or facts such as advice, suggestions, and feedback on actions. **Appraisal/Esteem support** is defined as the information that helps to promote self-evaluation and, in turn, skills, abilities, and intrinsic value. **Social network support** are the messages that help to enhance one's sense of belonging to a specific group with similar interests or situations.

There are those parents/guardians who do not fully understand HIV and its treatment and/or those who do not fully understand what social support entails nor its relevance. Hence, in treating AYALHIVs and increasing their self-management, parents'/guardians' understanding of the aforementioned must be addressed. It then follows that parents/guardians should be invited to participate in group sessions to discuss and learn about HIV and its lived reality. Should the need for individual therapy be identified, this should also be made available. Cross-referencing this with corresponding AYALHIVs' self-management may inform the utility of parent-based group sessions in the transition process.

Healthcare providers also play a role in the level of self-management that AYALHIVs demonstrate. It is important that healthcare providers are involved in the transition process – preparing AYALHIVs to leave paediatric care and initiating AYALHIVs to adjust to adult care. This may include timely preparation of patients (the why, when, and how of the transition process), the transfer of relevant information from the paediatric clinic to the adult clinic, as well as the education of AYALHIVs on the expectations of adult care. Interviewees also identified that some healthcare providers employ a doctor-centred communication style rather than a patient-centred style as some persons reported feeling like a statistic whilst in adult care, rather than a patient/person. It is imperative that healthcare providers, be them medical doctors, psychologists, social workers, or counsellors, allow room for AYALHIVs (and all other patients) to speak openly and feel comfortable as it promotes the dissemination of information and fosters a good relationship between AYALHIVs and their healthcare providers. The rapport between patients and healthcare providers impacts general adherence and self-motivation – facets of self-management.

Given the observed lack of self-management in the population, targeted interventions are recommended to boost self-management skills. All AYALHIVs will be asked to attend group therapy sessions focused on developing self-management skills (see Appendix 6 - Group Therapy Outline).

As aforementioned, parents and guardians will also be screened for depressive and anxious symptoms and invited to complete the PHQ-9 and the GAD-7 to assess the degree of experienced distress in the context of depression and anxiety. Should there be a need for further psychological services, individual therapy will be offered to both AYALHIVs and parents/guardians. Persons who score ≥ 10 on the PHQ-9 and GAD-7 (moderate depression / anxiety) will meet criteria for consideration of referral for the individual therapy.

PEER SUPPORT

Typical of the adolescent phase, peers can exert significant influence on each other as they work towards achieving peer acceptance. When utilised in a positive way peer influence can be very impactful. “Peer support” in the AYALHIV context allows for development of connectedness, role model and a sense of belonging in a space often marred by overwhelming stigma within the community. Peers living with HIV can share similar experiences with each other and mitigate against a sense of isolation, while modelling positive coping strategies (23).

Peer support programmes have been found effective in positively influencing AYALHIVs health-seeking behaviour and treatment outcomes (retention in care, adherence to ARVs, and viral suppression). Peer support may be done through in-person interaction or virtually through social media e.g., SMS text messages, WhatsApp. Programmes may also be local to the health facility or may extend into the community. The differing impact of location is uncertain.

An evidence-based approach to implementing peer support groups (23) should be adapted for use locally and is outlined in draft below.

1. Selection of peer supporters should be purposeful and must have defined selection criteria: Living openly with HIV¹, adherent with treatment plan and preferably a few years older than target group. Gender of the peer supporters should be selected with careful consideration of the target audience.
2. Peer supporters must be trained with skills-building efforts to enable them to effectively provide psychosocial support to AYALHIV, facilitate support groups, deliver psychoeducation about HIV, and recognize AYALHIV at risk.
3. The peer support programme should have defined training curricula and standards, with set goals/targets to achieve at the end of the support group.
4. Peer supporters must be included as a full member of the provider team and their activities should occur within the facility preferentially.
5. Peer supporters must receive supervision regularly and must be trained how to deal with challenging situations, e.g., threatening behaviour from AYALHIV.

¹ Intentional community disclosure will be a challenge to adopt locally.

COMMUNICATION

The recommendations globally and from our local experience underscore the importance of direct verbal communication between clinicians (in addition to the transfer summary shared as soft/ hard copy) for all AYALHIVs being transferred prior to transfer, but particularly for those high-risk AYALHIVs. Interim case conferences should be held on a case-by-case basis when deemed necessary for any challenging or potentially challenging case(s) by either party (adult or paediatric providers).

APPENDICES

APPENDIX 1. TRANSITION STATUS UPDATE

To be completed by provider at each visit; starting at age 11 yr.

	Not yet started	In progress	Completed	Comments
Transition manager identified				
Disclosure status				
Started discussion regarding transition				
Psychoeducation regarding illness				
Understands how illness affects body				
Understands how medications work				
Knows when and how to take medications				
Understands viral load and CD4 count				
Can give names and doses of current medications				
Can say if on 1 st , 2 nd line etc				
Knows most recent viral load				
Knows most recent CD4 count				
Can give 3-sentence summary of illness				
Group sessions				
Transition plan development with provider				
Self-management skills				
Making appointments				
Collects medications from pharmacy				
Communicates concerns to providers				
School/ vocational training plans				
Contraception/ family planning plan				
Understands impact risky behaviours has on illness				
Relevant referrals made				
Adult clinic				
Specific clinic identified				
Receiving adult provider identified				
Date to meet adult provider(s)				
Date to visit adult facility				

APPENDIX 2. TRANSITION CHECKLIST

(To be completed by provider 3-6 months before transfer to adult clinic)

Provider tasks <i>(tick If completed)</i>
€ New provider in adult clinic identified and informed of transfer
€ If high-risk case - direct verbal communication between paediatric and adult provider
€ Psychosocial support status of the client sufficient for transfer
€ Transfer summary completed
€ AYALHIV has copy of transfer summary
€ HIV literacy/ HIV understanding in AYALHIV sufficient for transition
€ HIV literacy/ HIV understanding in adults/ guardians sufficient for transition
€ AYALHIV agrees to transfer to adult clinic
€ Parent/ guardian ready for transition
Client Self-Management Competencies Achieved <i>(tick if completed)</i>
€ Knows name of medications and doses
€ Knows when and how to take medications
€ Adherent with medications consistently over prior 6 months
€ Follows through with investigations (viral load, CD4) over prior 6 months
€ Adherent with clinic visits over prior 6-9 months
€ Knows where and how to get medications, check correct and has collected them on at least 2 occasions
€ Able to discuss medications with HCP
€ Able to discuss health concerns with HCP
€ Can give 3 sentence summary of illness – diagnosis, medications, past hospitalizations, most recent viral load/CD4 count
€ Can schedule own appointments and has done so on at least 2 occasions (know where, when, and my provider/clinic)

APPENDIX 3. TRANSITION SUMMARY

(To be completed and shared with adult clinic prior to AYALHIVs first visit. Copy to be shared with AYA.)

Adapted from Transition Summary – PIDC, UHWI. R Pierre and J Moore

CLIENT INFORMATION	
CLIENT NAME:	REG #
DATE OF BIRTH (dd/mm/yyyy):	SEX: ☐ MALE ☐ FEMALE ☐ OTHER
ADDRESS:	TEL: CELL- HOME-
NEXT OF KIN:	RELATIONSHIP:
ADDRESS:	TEL: CELL- HOME-
REFERRING PHYSICIAN	
NAME	TEL:
EMAIL:	CLINIC NAME

DATE OF HIV DIAGNOSIS (mm/yyyy):	MODE OF TRANSMISSION: MTCT ☐ BEHAVIORAL (CONSENSUAL) ☐ (NON-CONSENSUAL) ☐ OTHER ☐
----------------------------------	--

DIAGNOSES AND CURRENT PROBLEMS	PROBLEM DETAILS AND RECOMMENDATIONS
Primary Diagnosis (CDC Category)	
Secondary Diagnoses	
○ Behavioural	
○ Communication	
○ Hearing/Vision	
○ Learning	
○ Orthopaedic/Musculoskeletal	
○ Physical Anomalies	
○ Respiratory	
○ Sensory	
○ Feed & Swallowing	
○ Other	

ANTIRETROVIRAL DRUG HISTORY

ANTIRETROVIRAL REGIMEN	DATE STARTED (mm/yyyy)	REASON FOR SWITCHING (if applicable)

PROBLEM LIST (MEDICAL HISTORY)

<u>CURRENT PROBLEMS</u>	<u>PAST AND RESOLVED PROBLEMS</u>

CURRENT MEDICATIONS & OTHER THERAPIES

RECENT INVESTIGATIONS

	Value	Date (dd/mm/yyyy)
HIV VIRAL LOAD		
CD4		
% CD4		
HB		
WBC		
PLATELET COUNT		
ELECTROLYTES Normal/Abnormal	(If abnormal)	
RENAL FUNCTION Normal/Abnormal	(If abnormal)	
LIVER FUNCTION Normal/Abnormal	(If abnormal)	
OTHER		

PSYCHOSOCIAL

	PROBLEM DETAILS AND RECOMMENDATIONS
DISCLOSURE (DATE/AGE)	
CURRENT REPORTED ADHERENCE	
SELF-MANAGEMENT STATUS	
PERSONALITY PROFILE • communication strategies that work best for this client • unique characteristics - strengths, achievements, hopes for the future and goals; challenges	
SOCIAL NEEDS	

Emergency contact (Name/Mobile #)	
Emergency care plans	

FORM COMPLETED BY:

Signature:	Date:
Name:	
Consultant Paediatrician	Date
Signature:	
Name	

NEW PATIENT - ADOLESCENT

Ward of State? Yes () No () *If yes, Date Assigned* _____

UHWI [] STH [] BHC [] CHC [] Date of Visit: _____

Adolescent Sex Male [] Female [] Docket # _____

Last _____ First _____ Middle _____ DOB _____ Age _____

Occupation _____ Code _____

Employed? Yes () No () Cell Phone _____

Marital	Single () Married () Divorced ()
Status	Widowed () Visiting () Common Law ()

Mother () / Guardian () Relationship _____ Home Phone _____ Cell Phone _____

Last Name _____ First _____

Mailing Address _____ Community _____ Parish _____

Health Status of Mother : Alive/Well (), Alive/Sick (), Unknown (), Dead (), Death date _____

Father Last Name _____ First Name _____

Health Status of Father : Alive/Well (), Alive/Sick (), Unknown (), Dead (), Death date _____

CC: _____

History of Presenting Complaints:

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, 1st Draft: November 13, 2002, Sept. 28, 2009

NEW PATIENT - ADOLESCENT

Date of Visit: _____ Referred by: _____

Childhood History

1. Immunizations DPT/OPV(IPV) [], MMR [], HIB [], HEP. B []

2. Development: Normal [], Abnormal []

3. Childhood Illnesses:

4. Medication Allergies/intolerance:

5. Past Surgical History:

6. Blood Transfusions:

1. Date _____ Hospital _____, 2. Date _____ Hospital _____,

3. Date _____ Hospital _____, 4. Date _____ Hospital _____,

7. Past Obstetric History: Parity _____, Gravida _____ FT _____ PT _____ Sab _____ Tab _____ LC _____

Sexual History

8. Sexual Abuse: Yes (), No (). If Yes please specify _____

9. Sexual Orientation: Heterosexual (), Homosexual (), Bisexual (), Asexual () Questioning ()

10. Age sexual intercourse started: _____, 11. Was first sexual intercourse forced? Yes () No (), 12. No. of sex partners: _____
Sexual partners male () female () both ()13. Mode of HIV Transmission: () MTCT, () Sexual – forced, () Sexual – consensual, () Blood Transfusion,
() Shared Needle, () Other _____14. Any HIV high risk behavior: () Prostitute related, () Promiscuity, () Past STD infection, () Unprotected Sex,
() Imprisonment, () Drug Use, () Other _____

15. Family Planning Method (s): 1. _____, 2. _____, 3. _____

Contraception: Abstinence () Withdrawal () Condoms () Pill () injection () IUD () Subcutaneous rods () other _____

Condom used at last sex? yes () no ()

16. **STI History:** _____

17. **HIV Disease History:**

18. Current Medications:

19. ARV Treatment:

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, 1st Draft: November 13, 2002, Revised Sept. 28, 2009

NEW PATIENT - ADOLESCENT

Last Name _____ First _____ Docket # _____

Children**Mother**

DOB Age Sex Status Care

Father # 1

Father #__

Father #__

Social History

1. Guardianship: _____
2. Household Members: _____
3. Disclosure Issues: _____
4. Housing: _____
5. Financial: Income: _____ Expenditure: _____
6. Insurance: _____
7. Education: _____
8. Substance abuse: _____
9. Non-medical needle use or tattoos: _____

URINALYSIS

Leukocytes	Nitrite	Uro-Bilinogen	Protein	PH	Blood	Specific Gravity	Ketone	Bilirubin	Glucose

REVIEW OF SYSTEMS

N A	System
	General
	Eye
	ENT
	CV
	Resp
	GI
	GU
	MS
	Skin
	Neuro
	Endocrine
	Heme
	Immune
	Other
	Development

Doctor's Signature _____ Date _____

Last Name _____ First _____ Docket # _____

Psychosocial review

Transition	Can you explain the important things about your condition in three or four sentences? How much of your health care is your responsibility? What do you need to do to get ready for adult health care? What kind of services will you need to find in the adult system?
Home	Where do you live? Who do you live with? How well do you get along with the people you live with? Anyone at home you can talk to? Recent additions/losses? Are there any issues with housing and your health? (e.g., accessibility) Do you have concerns about how your illness affects your family's function? Who knows about your illness? (family members/friends) If you live in more than one home, are there differences in the way you take care of your illness in the two homes?
Rx – Medication and Treatment	What medications do you take? What are the dosages? (can they answer these questions? Yes ☐ No ☐) Who is in charge of your medication? What is your role? How do your medications/treatments fit in your routine during the week/weekend? What happens when you miss a dose? What do you like and dislike about your medication?
Education, Eating, Employment	Do you go to school? What school? What grade are you in? Ever skipped/repeated a grade? Are you in a special class? Friends at school? Recent changes in school? Ever suspended/expelled? Adults at school you can talk to? Are you working? Where? How much? Future plans for work/school? Does your illness make things different or difficult for you at school? Is there someone you can talk to or go see for help/support at school? How often do you have to miss school because of your illness? Any recent changes in your weight? Have you dieted in the past year? How often? Exercise? Anything else to manage your weight? What do you think is a healthy diet? How does yours compare? What do you like/dislike about your body? How does your condition or medications affect your diet and appetite? Does your condition affect how your body looks? In what way?

Activities, Affect	<i>Do you have friends?</i> <i>What do you and your friends do for fun?</i> <i>What do you and your family do for fun?</i> <i>Extracurricular activities?</i> <i>Hobbies?</i> <i>Media time/internet safety?</i> Does your condition get in the way of participating in activities that your friends do? Do you have friends or talk to people who have similar health/disability issues? Do you use the internet to connect with other teens with health conditions or learn about your conditions? Does your condition affect your sleep? Does your condition make you sad, angry, or anxious? Have you ever had suicidal thoughts or tried to hurt yourself because of your condition?
Drugs	<i>Do any of your friends smoke/use alcohol or other drugs?</i> <i>Do you smoke? Drink? If yes, how much and how often?</i> <i>Anyone in your family smoke/drink/use other drugs?</i> <i>Drink or use other drugs alone?</i> <i>CRAFFT questions</i> What do you know about the effect of alcohol/cigarettes/street drugs on your condition or treatments? Do you ever use alcohol/cigarettes/street drugs to treat your condition, symptoms or medication side effects?
Sexuality, Safety, Suicide, Sleep	Are you/have you been in a romantic relationship? Have you ever had sex? Are you using birth control? Do you use condoms every time? Have you ever been pregnant/gotten someone pregnant? Did someone ever make you do something you didn't want to do? How many partners have you had? Have you ever had an STI? What do you know about the effect of your condition on your sexual health? Are there any limits on contraception you can use because of your condition? Have you talked about the genetics of your condition? (i.e., it can be passed on to your children?) Do you ever feel sad or down more than usual? What do you do to cheer yourself up? Do you ever think a lot about or want to hurt yourself/someone else? Have you ever tried to kill yourself? Do you have anyone to talk to? Who? Trouble sleeping? How difficult to fall asleep/stay asleep?

NEW PATIENT - ADOLESCENT

Adolescent: Last Name _____ First _____ Docket # _____

PHYSICAL EXAM				
Height cm	Weight kg	SA m ²	Temp. °C	P
%	%		TM/O/R/Ax	R
				BP

NA	
Gen. Appearance	
Facies	
EYES	
Conj. / lids	
Pupil / iris	
Fundi	
ENMT	
External	
EAC/TM	
Hearing	
Nose	
Lips/teeth/gums	
Oropharynx	
Neck-general	
Thyroid	
Parotid gland	
Clubbing finger/toe	

NA	
RESPIRATORY	
Effort	
Percus. / palp.	
Auscultation	
CARDIOVASC	
Palpation	
Auscultation	
Pulses	
Breasts	
GI	
Abdm	
Liver	
Spleen	
GU	
Rectal	

NA	
LYMPH	
Epitrochlear	
Neck	
Axilla	
Groin	
MS	
Station/gait	
Digits	
Joints	
Muscles	
Skin	
NEURO	
CN	
Motor	
Sensation	
DTR	
Prim reflex	
Development	
DDST.	

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, † Draft: November 13, 2002, Revised Sept. 28, 2009

NEW PATIENT - ADOLESCENT**Adolescent** Last Name _____ First _____ Docket # _____**Assessment**

Diagnosis	ICD Code
1. HIV/AIDS class:	
2.	
3.	
4.	
5.	

PLAN:

R	N	
		PURPLE
		CBC & Diff
		HIV Rapid test
		PCR: DNA/RNA
		Standard Viral load
		CD4 Panel
		YELLOW
		T-cells
		HIV Culture
		Storage
		SPECIAL
		Research
		RED TOP
		Na*, K*, Cl*, HCO
		BUN, creat
		Glucose
		AST, ALT, Alk ph, Bili
		Ca ²⁺ , PO ₄ ²⁻
		Amylase
		Lipase
		Cholesterol
		Triglycerides
		Albumin/TP
		SEROLOGY (REDTOP)
		HIV ELISA
		VDRL
		Hep C antibodies
		Hep B Ab / HBsAg
		HTLV -1 ELISA
		URINE
		U/A
		Urine culture
		CULTURES
		RADIOLOGY

IMMUNIZATIONS	Initials	FOLLOW UP
		Purpose
		CONSULTS
		COUNSELING
Mantoux		HIV exposure/risk
VISION SCREEN		Diet
HEARING SCREEN		Safety
MEDICATION TEACHING		Illness
		Development
		Universal precautions
		Other:

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, 1st Draft: November 13, 2002, Revised Sept. 28, 2009

APPENDIX 5. FOLLOW-UP ADOLESCENT PATIENT – CLINIC INTAKE FORM

Page 1 of 6

ADOLESCENT CLINIC FOLLOW -UP FORM

Site _____ Sex Male [] Female []

Date of Visit: _____ Visit type: () Follow -up, () Well Child, () Sick, () Study, () Insurance

Adolescent

Last _____ First _____ Middle _____ Docket # _____ KPAIDS# _____

DOB _____ Age _____ Wks (), Mths (), Years () Ward of State? Yes () No () If yes, Date Assigned _____

Mother () / Guardian () Relationship _____ Home Phone _____ Cell Phone _____

Last Name _____ First _____ KPAIDS # _____ Docket # _____

Mailing Address _____ Community _____ Parish _____

Health Status of Mother : Alive/Well (), Alive/Sick (), Unknown (), Dead (), Death date _____ Cause _____

Father Last Name _____ First Name _____

Health Status of Father : Alive/Well (), Alive/Sick (), Unknown (), Dead (), Death date _____ Cause _____

Who brought child to the clinic? Surname _____ FirstName _____ Relationship _____

Antiretroviral Dosage/Frequency 1. _____ 2. _____ 3. _____ Prophylaxis Dosage/Frequency 1. _____ 2. _____	Other Medications Dosage/Frequency 1. _____ 2. _____ 3. _____ 4. _____ 5. _____ 6. _____
--	---

Social History

- Guardianship: _____
- School / Daycare: _____
- Employment : _____
- Substance abuse: _____
- Sexual/Contraception _____

Past Medical History:

Illnesses: _____

Hospital/ER/Surgery: _____

Diet: _____

Immunization review: _____

REVIEW OF SYSTEMS

N	A	System
		General
		Eye
		ENT
		CV
		Resp
		GI
		GU
		MS
		Skin
		Neuro
		Endocrine
		Heme
		Immune
		Other
		Development

Dr.'s/Nurse's Signature _____ Date _____

Adapted from University of Maryland with permission, # Draft: November 13, 2002, Revised Sept. 2009

Last Name _____ First _____ Docket # _____

Chief Complaint: _____

History of Presenting Complaint(s)

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, ‡ Draft: November 13, 2002, Revised Sept. 2009

Last Name _____ First _____ Docket # _____

PSYCHOSOCIAL REVIEW

Transition – Can they explain the important things about their condition in three or four sentences? How much of their health care are they taking responsibility for (remembering meds; making appts etc)?

Home – ask about relationships with each member of household, ie how do you get along with.....; do you think your parent understands you; cares about you; listens to what you have to say/ Do you have concerns about how your illness affects your family's functions? Who knows about your illness? (family members/friends)

Rx – Can they name their medications and give the doses? Who is in charge of medication? What happens when you miss a dose? What do you like and dislike about your medication?

Education – School name, grade, have you had to repeat a grade; what are your grades on average, ever been suspended or expelled, how do you get on with your teachers? Classmates? Any bullying? etc
Does your illness make things different or difficult for you at school? Is there someone you can talk to or go to see for help/support at school? How often do you have to miss school because of your illness?

Eating – What is their intake like – how many meals? Snacks? How do they feel about their body? Weight? Are they doing anything to change weight? If yes then what? Does the illness/medications affect their diet and appetite? Do they think the illness affects how their body looks? In what way?

Employment – Are they working? Where? How much? Any future plans for work/school?

Activities – do they have any close friend that they trust? What do they do when they hang out with friends? Extracurricular activities? Media use? Does the illness get in the way of participating in typical activities? Do they use the internet to learn about your condition or talk to other people about it

Drugs – friends use? Personal use? Smoke/vape/edibles/alcohol/other??

Sexuality and sex – orientation, 1st intercourse, # partners, type partners, condom use, risk behaviours, forces/coerces sex etc? Contraception?

Safety– abuse concerns, etc.

Suicide/Mood–how do they usually feel? Do they ever feel so down that they think of killing themselves? DO they ever think a lot about hurting someone else? Have they ever tried to kill themselves? Any of these thoughts/actions because of their illness? Who do they have to talk to?

Sleep–How difficult is it to fall asleep/stay asleep? Does the illness affect their sleep?

Doctor's Signature _____

Date _____

PHYSICAL EXAM

PHYSICAL EXAM							
Height cm	Weight kg	Head C. cm	SA m ²	Temp. °C/°F	p	R	BP
%	%	%		TM/O/R/Ax			

[illegible]

Clubbing finger/toe

[illegible]

DDST.

[illegible]

Protein: Sugar : Blood:

Rec

☐ Thyroid

☐ Spine (Scoliosis)

Adapted from University of Maryland with permission, † Draft: November 13 2002. Updated 2014 to include MBHC

CLINIC FOLLOW-UP FORM

Adolescent Last Name _____ First _____ Docket # _____

Assessment**PLAN:**

Diagnosis	ICD Code
1. HIV/AIDS class:	
2.	
3.	
4.	
5.	

R	N	
		R-Requested, N-Not Requested
		PURPLE
		CBC & Diff
		HIV Rapid test
		PCR/DBS
		Standard Viral load
		CD4 Panel
		YELLOW
		T-cells
		HIV Culture
		Storage
		SPECIAL
		Research
		RED TOP
		Na ⁺ , K ⁺ , Cl ⁻ , HCO ₃ ⁻
		BUN, creat
		Glucose
		AST, ALT, Alk ph, Bili
		Ca ²⁺ , PO ₄ ²⁻
		Amylase
		Lipase
		Cholesterol
		Triglycerides
		Albumin/TP
		SEROLOGY (REDTOP)
		HIV ELISA
		VDRL
		Hep C Ab
		Hep B Ab / HbsAg
		HTLV -1 ELISA
		URINE
		U/A
		Urine culture
		CULTURES
		RADIOLOGY

IMMUNIZATIONS	Initials	FOLLOW UP
		Purpose
		CONSULTS
		COUNSELING
		HIV exposure/risk
		Diet
		Safety
		Illness
		Development
		Universal precautions
		Other:
PPD		
HEARING SCREEN		
VISION SCREEN		
MEDICATION TEACHING		
Letter to primary MD		

Doctor's Signature _____ Date _____

Adapted from University of Maryland with permission, 1st Draft: November 13, 2002, Revised Sept. 2009

APPENDIX 6. GROUP THERAPY OUTLINE

BRIEF OVERVIEW

Adolescents living with HIV are vulnerable to various physical, emotional and social challenges that impact their ability to self-care. Psychosocial interventions focus primarily on medication adherence while neglecting the other aspects of care (24). Research shows that adolescents are rarely consulted about decisions regarding their care therefore negating an adolescent-centred intervention (25). Psychosocial challenges directly and indirectly affect levels of care which results in an increased risk of AIDS-related mortality. The need for culturally appropriate interventions which address adolescents' self-care needs, challenges and preferences is imperative (24). Self-management skills are positively associated with successful health outcomes among adolescents living with HIV. Therefore, interventions that target and improve these areas improve the quality of life of the adolescent. Group therapy is one method used to improve these skill sets, it is an approach which offers several benefits including giving the adolescents a space to safely work through their challenges, to recognize that other adolescents also share similar challenges which reduces feelings of loneliness and increases a sense of belonging among others. These fourteen group activities cover a range of topics including knowledge and awareness, medication adherence, disclosing status to friends/family, self-management, coping with stress and anti-stigma. Sessions will be led by the (associate) psychologist serving the paediatric based clinic.

1. ORIENTING MEMBERS TO THE GROUP

Learning Objectives

- Introduce members to the processes of group therapy
- Roles and expectations

2. NORMALISING HIV

Learning Objectives

- To get young people to realise that they are not the “only ones” living with HIV
- Build emotional resilience
- Have more open discussion about one’s status that is stigma free

3. CHOICE OF LIFE PILL

Learning Objectives

- To get young people to become aware of their autonomy in taking their meds
- Become aware of obstacles and facilitators for taking meds

4. WHAT MATTERS TO ME

Learning Objectives:

- Explore the things that are important to adolescents.
- Assist them in making connections between what is important to them and keeping them well.
- Help them make the connection between their medication and them being there for the things that are important in their lives.

5. THE HONESTY JAR MEDICATION WORKSHOP

Learning Objectives:

- Understanding the how medication works
- Understanding the barriers and benefits to taking medication

6. PAST, PRESENT AND FUTURE

Learning Objectives:

- Work on bringing awareness to where adolescents tend to reside in their minds
- Teach mental flexibility and its benefits
- Help adolescents to realise that they can choose a more helpful option by being in the present and aspire to future goals

7. MINDFULNESS EXPERIENCE - STRESS MANAGEMENT

Learning Objectives:

- Explore ways of managing overwhelming emotions
- Help adolescents to learn to become aware of their focus and develop strategies to shift their focus to more helpful outcomes

8. THE MASKS WE WEAR

Learning objectives:

- Speak openly and freely about HIV
- Learn more about what living with HIV is like

9. SCHOOLS CHALLENGE QUIZ

Learning objectives:

- Increase members’ knowledge and understanding of HIV
- Bring awareness to difficult topics

10. MY PIT OF STRUGGLES

Learning objectives:

- Bring awareness to what it means to live with HIV
- Help participants to learn that they all have struggles, and the commonalities that exist.
- Help ALHIV find and create a healthy lifestyle when living with HIV

11. TRUST AND DISCLOSURE

Learning objectives:

- Become aware of the importance of trust.
- Understand the risks and benefits involved in disclosing.
- Explore the possibilities of developing a supportive community.

12. THE MANY SIDES OF ME - THOUGHTS, FEELINGS AND BEHAVIOUR

Learning Objectives:

- Gain an understanding of thoughts, feelings, actions and physiology.
- Process how negative reactions of others to one's HIV status might affect them.
- To develop coping tools on how to respond.

13. ANTI-STIGMA SOUNDTRACK

Learning Objectives:

- Gain an understanding of stigma and HIV
- Think about ways to educate and change people's perceptions of people living with HIV.

14. FINAL GROUP - CELEBRATION CEREMONY

Learning Objectives:

- Gain closure and reflection
- Highlight the importance of community

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